televëda

APRIL 2026

UN REACHED

Social Isolation Among Rural Older Adults, Veterans, and Native American Communities

Why the systems built to address it have consistently failed to reach them

285

VETERANS REACHED IN 2024 PILOT

100%

REPORTED FEELING MORE CONNECTED

Draft

The webpage is the updated version

IN COLLABORATION WITH

U.S. DEPARTMENT OF VETERANS AFFAIRS

ARIZONA DEPARTMENT OF VETERANS' SERVICES

NAVAJO NATION VETERANS ADMINISTRATION

ARIZONA DEPARTMENT OF HEALTH SERVICES

NAVIGATION

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SOURCING STATEMENT

On Evidence and Honesty

This report cites primary sources throughout. Where the evidence is strong, we say so. Where it is emerging, we say that too. Where we draw on program evaluation data rather than independent research, we identify it as such.

Every statistic appears with its source in the same sentence. The field has developed a habit of citing aggregates without origins. This report does not participate in that habit.

We distinguish throughout between peer-reviewed longitudinal evidence, government survey data, program evaluation findings, and participant self-report. These are different kinds of evidence. They warrant different weight. We have tried not to flatten that distinction in the service of a cleaner argument.

Where data carries significant methodological limitations, those limitations are stated in the body text. Where a figure should be read as indicative rather than definitive, we say so.

CONFLICT OF INTEREST

Disclosure

This report was produced by Televeda. Section Five presents program evaluation data from Televeda's own Arizona partnerships.

That data is identified throughout as internal evaluation evidence, not independent research. All other findings draw on published primary sources, cited directly.

We have tried to present the evidence honestly, including where it is thin, where studies are small, and where long-term outcomes remain unknown. We have tried not to use the strength of the published evidence to paper over the limitations of the implementation evidence, or vice versa.

Readers evaluating program partnerships with Televeda should weigh Section Five accordingly as the most specific data currently available for these populations under these conditions, presented with the limitations named.

EXECUTIVE SUMMARY

Five Findings on a Crisis the System Was Not Built to See

> *She is seventy-one years old. Agnes lives on the Navajo Nation in northeastern Arizona, where the average drive to a healthcare facility runs three to four hours on roads that flood in spring and ice over in winter. She served in the Army. She carries a service-connected disability. She has not seen a VA provider in more than a year. She does not appear in any social isolation dataset, in the literature on Veterans' health, on Native American health, or on rural older adults, because the systems that generate that data were not built to reach her.*

4 in 10
Americans 45+ report loneliness
AARP Loneliness Survey, Dec 2025

+5pts
Rise in loneliness prevalence since 2018 after 8 flat years
UCLA Loneliness Scale, n=3,276

15
Cigarettes per day is the comparable mortality risk to social isolation - U.S. Surgeon General, 2023

FIVE KEY FINDINGS

01 The crisis is accelerating and the trajectory matters more than the snapshot. Adults aged 45 to 59 are now lonelier than those over 60, upending the assumption that has organized this field for two decades: that social isolation is primarily a condition of later life. For many people who will reach 70 struggling with isolation, the damage began forming in middle age.

02 The health consequences are not marginal. Social isolation is associated with a 29 percent increased risk of coronary heart disease and a 32 percent increased risk of stroke, in Valtorta and colleagues' systematic review of 23 longitudinal studies published in Heart in 2016. The longer isolation goes unaddressed, the worse the physiological outcome.

03 The populations bearing the highest burden are the ones current programs most consistently fail to reach: older adults in rural communities, Native American elders, Veterans particularly those from post-9/11 service eras, and women Veterans who experienced military sexual trauma during service. Section Three names why.

04 Group-based virtual programming produces measurable, consistent improvements in social support, anxiety, and depression among Veteran and older adult populations. For people who cannot make a multi-hour drive to a VA facility, virtual programming does not substitute for in- person care. It makes care possible where in-person care cannot reach.

05 The gap is not evidence. The evidence exists. The gap is implementation: funding directed at the right models, cultural adaptation that most existing programs have not achieved, and a willingness to design for the people who have been left outside the frame.

01

SECTION ONE

The People the Data Does Not See

The social isolation epidemic is not spread evenly. It concentrates among the populations least served by the systems built to address it.

A Distinction on Which This Report Depends

Social isolation is an objective condition, the measurable absence of social contact and relationship. Loneliness is something different: a subjective experience of disconnection that can exist even in the presence of others, and can be absent among people whose social networks are objectively sparse. The two are related but distinct, they require different measurement, and they respond to different interventions.

The three-item UCLA Loneliness Scale is widely used as a clinical screen for subjective loneliness; a positive result is a flag for follow-up, not a diagnosis. The PROMIS Social Isolation measure captures objective isolation. Both appear in this report because the populations it describes often suffer from both simultaneously in ways standard screening does not capture and existing programs were not designed to address.

Who Is Lonely and How the Numbers Are Moving

Adults aged 45 to 59 are now lonelier than adults over 60. This finding, from the December 2025 AARP survey tracking the same population across 2010, 2018, and 2025, inverts an assumption that has shaped this field for two decades: that social isolation is primarily a condition of later life, accelerated by bereavement, retirement, and reduced mobility.

What the data shows instead is isolation taking root in midlife, shaped by caregiving pressure, workforce transition, and the gradual erosion of the community anchors that once held social lives together without anyone calling it an intervention.

LONELINESS RATE, ADULTS 45+

35% → 35% → 40%

2010 2018 2025

Flat for 8 years, then accelerated. AARP Social Connectedness Survey (Choi-Allum & Rainville, 2025), UCLA Loneliness Scale, n=3,276

LONELINESS BY GENDER, 2025

42% Men — 17% have no close friends

37% Women — 13% have no close friends

Men now report higher loneliness rates than women for the first time. AARP, 2025

A global pandemic severed social ties at precisely the age when they are hardest to rebuild. Social media expanded during those years, producing the sensation of connection without its substance. Religious communities, civic organizations, neighborhood associations, the institutions that once generated social contact as a by-product of participation declined further. None of this is going to reverse on its own.

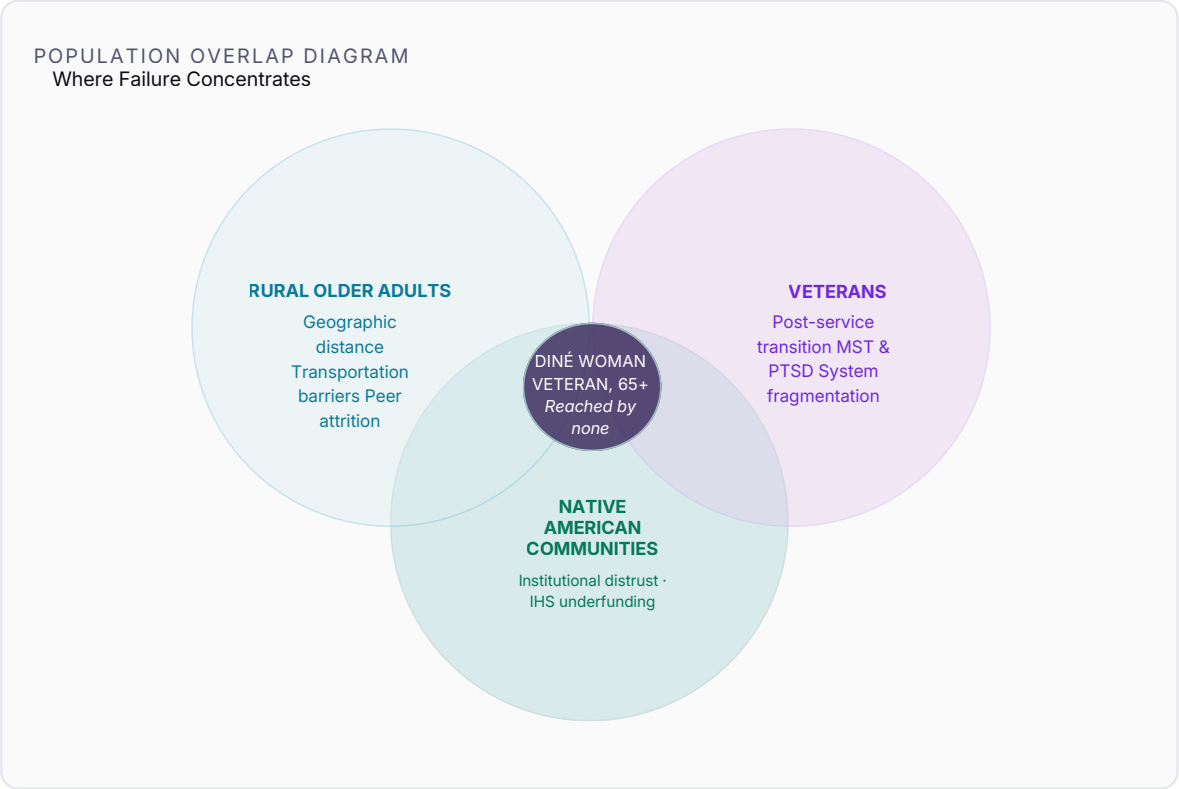
The Populations Bearing the Highest Burden

Rural older adults face compounding structural barriers that urban program models were not built to cross. Distance from service providers. Limited and unreliable transportation. Chronic health conditions that restrict mobility before anyone has rebuilt a social infrastructure around that restriction. In communities where the nearest senior center requires a forty-minute drive that becomes impossible the day you can no longer drive, these are the permanent conditions within which any program must operate. The program must be built to cross them; they will not adjust to accommodate the program.

Veterans carry a specific version of this problem. Social isolation among Veterans does not begin in old age, it begins in the years after separation from service, when the structured community of military life dissolves and the transition into civilian social networks proves harder than the transition itself. The 2023 CONNECTED study (Eliacin et al.) found 39% of surveyed Veterans reported significant social isolation, with rates notably high among Veterans under 55.

Women Veterans carry a distinct burden within this one. One in four women Veterans reports having experienced military sexual trauma during service, a figure likely undercounted. PTSD rates among this population are nine times higher than among Veterans without that history (Galovski et al., 2022). Women Veterans are the fastest-growing VA demographic and among the hardest to retain in standard social programming.

Native American communities face structural disadvantage so historically produced that the generic framing of health disparity does not serve it. On the Navajo Nation, the average drive to a healthcare resource runs three to four hours. More than 73% of households lack broadband access. The federal government operated the Phoenix Indian School in Arizona from 1891 to 1990, one hundred years of explicit policy designed to sever the relational systems through which Diné people understand health, kinship, and community.



She is simultaneously eligible for services from multiple systems and effectively reached by none of them

02

SECTION TWO

What Isolation Actually Costs

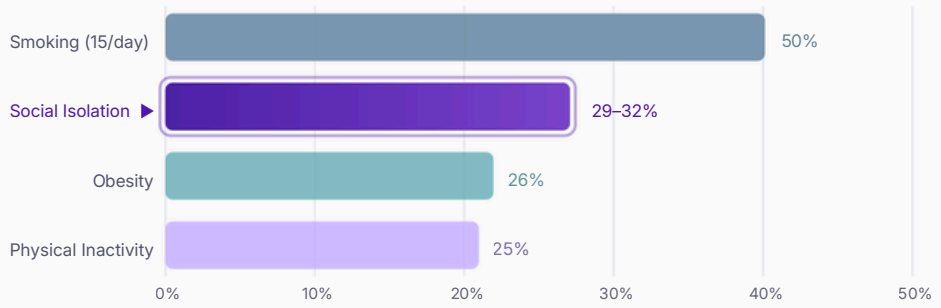
Thehealth consequencesand the financial consequences are the same argument. Separating themhas allowed health systems to treat the first as a clinical concern and the second as someone else's problem.

Social isolation increases the risk of dying early by roughly 30 percent, a figure that has held up under meta-analytic scrutiny across four decades and millions of participants. Wang and colleagues' 2023 meta-analysis of 90 prospective cohort studies involving 2.2 million individuals, published in Nature Human Behaviour, found social isolation associated with a 32 percent increased risk of all-cause mortality and a 34 percent increased risk of cardiovascular disease mortality. The foundational Holt-Lunstad, Smith, and Layton meta-analysis in PLOS Medicine in 2010, covering 148 studies and 308,849 participants, established that social relationships influence mortality risk at a magnitude comparable to smoking, physical activity, and obesity. Five years later, Holt-Lunstad's follow-up in *Perspectives on Psychological Science*, covering 70 studies, put the odds ratio for social isolation and mortality at 1.29.

COMPARATIVE MORTALITY RISK

Social Isolation vs. Other Major Risk Factors

Premature mortality risk increase relative to comparison group(approx)



Sources: Holt-Lunstad et al., *PLOS Medicine , 2010 (148 studies, 308,849 participants);* Office of the Surgeon General, 2023

The cardiovascular evidence is specific enough to stand on its own. Valtorta and colleagues' 2016 systematic review in Heart, covering 23 longitudinal datasets and more than 181,000 adults followed for up to 21 years, found poor social relationships associated with a 29 percent increased risk of coronary heart disease and a 32 percent increased risk of stroke. The effect size is comparable to anxiety and job strain, both of which are already treated as recognized cardiovascular risk factors in clinical guidelines, while social isolation is not. The 2023 Surgeon General's advisory on loneliness drew the comparison to tobacco directly. Most health systems acknowledged the advisory and adjusted nothing in their operational model.

Duration compounds everything. Chen et al.'s 2024 analysis in the *Journal of Epidemiology and Global Health* found that persistent, unresolved isolation carries higher cardiovascular and mortality risk than temporary or episodic isolation. The risk accumulates. For the person who has been isolated for a decade by the time a health system notices she is not showing up, the clinical question has moved well past prevention.

The Financial Case

Social isolation among older adults is associated with an estimated $6.7 billion in additional annual Medicare spending, based on Flowers et al. (2017) analysis of Medicare claims from 2006 to 2012. Socially isolated beneficiaries were also more likely to require skilled nursing facility care, the highest-cost service in the Medicare portfolio. This cost concentration indicates excess spending is driven by higher-acuity service utilization rather than marginal increases in routine care.

The per-person cost differential is $1,608 per year: the additional annual Medicare expenditure for a socially isolated beneficiary compared to a socially connected peer. Socially isolated older adults were significantly more likely to require skilled nursing facility care, the highest-cost service in the Medicare portfolio, and they cost the program more when they did.

Two points about these figures. First, they are derived from data now more than a decade old. Loneliness prevalence has increased in more recent estimates; higher prevalence is likely to increase aggregate cost, suggesting the $6.7 billion figure is a floor. Second, the national aggregate is less useful for local investment decisions. The per-person differential is the more relevant metric for budget holders.

WORKED EXAMPLE · LOCAL COST MODELING

A regional health system with 8,000 Medicare beneficiaries aged 65+, assuming a 25% social isolation prevalence consistent with the literature, faces roughly $3.2 million per year in excess Medicare spending attributable to isolation.

If a virtual group intervention reaching 500 of those beneficiaries costs $400,000 annually to operate and reduces isolation-related utilization by even 15%, the cost offset exceeds the program cost in the first year.

These are illustrative figures, not a projection but the math is available to any health system willing to apply its own numbers.

The cognitive trajectory mirrors the cardiovascular one. Chronically isolated older adults face elevated rates of dementia and cognitive decline, with neurobiological mechanisms documented by Hawkley and Cacioppo (2010). Social isolation is now recognized in the dementia prevention literature as a modifiable risk factor. Primary care has not yet integrated this into standard screening or referral.

The cost burden falls hardest on the systems least able to absorb it. Tribal health programs operating under the IHS face chronic under-resourcing relative to the populations they serve. Isolation increases healthcare utilization, and tribal health systems bear that increased utilization without the funding base to address its causes.

> *The financial pressure of untreated isolation compounds the structural disadvantage that produced the isolation in the first place. In practice, this means health systems are systematically spending more on late-stage intervention in the same populations where early, lower-cost prevention is least available.*

$6.7B
Est. excess Medicare spend/yr
Flowers etal.,AARP2017

$1,608
Additional cost per isolated beneficiary per year

+5pts
Loneliness prevalence rise since 2018 → floor is now higher

03

SECTION THREE

A Design Failure, Not a Resource Failure

The failure to reachrural older adults, Veterans, and NativeAmerican elders runs deeper than resource constraints. Recognizing that distinction is the prerequisite for fixing it.

Most social isolation interventions in the United States were designed for a specific person: middle- class, English-speaking, technology-comfortable, within reasonable distance of a healthcare provider, and navigating systems that were built, however imperfectly, with their needs in mind. That person exists. They are also not who this report is about.

> *"A program that was not built for you is not a program you failed to use. It is a program that failed to reach you."*

The assumptions embedded in most programs are specific enough to name. They presume participants who can attend in person, navigate digital platforms without assistance, read program materials in English, and trust the institutions delivering the service. Strip away any one of those assumptions and the program stops working. Strip away all four and the person it was supposedly designed to serve cannot get in the door.

Health Is Not an Individual Condition

Hózhó is the Diné framework for health, balance, and beauty. The state of being in right relationship with one's self, family, community, land, and the living world. Kahn-John (Diné) and Koithan describe it in their 2015 peer-reviewed analysis in *Global Advances in Health and Medicine* as the complex wellness philosophy and belief system comprised of wellness principles that guide one's thoughts, actions, behaviors, and speech. A person in hózhó is whole within their relational web. Disruption of those relationships is the health problem. Restoration of those relationships is the intervention.

K'é, the Diné kinship system through which hózhó is understood and maintained, is the architecture of social life on the Navajo Nation. A person's clan relationships determine their obligations, their belonging, and their place in the web of reciprocal connection that constitutes social existence. A person separated from those relationships is structurally unwell in a way that English does not have a word for.

Televeda's talking circle model was built in direct consultation with Diné community leaders, traditional healers, and narrative therapists because of this. When a participant in a Navajo Nation Talking Circle introduces themselves, the platform displays their name, tribe, and parents' clan. The k'é greeting that anchors every Diné social encounter. A virtual speaking token passes among participants as is customary in in-person circles. The form carries the function.

THE DESIGN INCOMPATIBILITY

A wellness program translated into Diné Bizaad but retaining the conceptual framework of individual psychological wellbeing arrives with untranslated assumptions. It presumes the individual is the unit of intervention; hózhó and k'é presume the relationship is. That incompatibility cannot be corrected by better translation. It requires a different design from the ground up.

The Trust Deficit Has a Specific History and a Specific Author

The Phoenix Indian School operated in Arizona from 1891 to 1990. For nearly a century, the federal government separated Diné children from their families, prohibited them from speaking their language, and stripped away the clan names through which a Diné person understands who they belong to. This was not incidental to the school's purpose. It was the purpose.

The federal government dismantled k'é as explicit policy for one hundred years. The elders these programs are now trying to serve lived through the end of that era. Their parents or grandparents may have attended that school. The VA and the IHS are federal institutions. Programs that treat the resulting distrust as a messaging problem solvable with better outreach copy have misread the situation entirely.

> *"We are very particular about not being parachute droppers, because historically, that has been the case with all nonprofits."*
> MAYANK MISHRA, CO-FOUNDER, TELEVEDA

Trust here is rebuilt through sustained presence, through genuine community co-design rather than community consultation, through demonstrated consistency over years rather than program cycles. Facilitators whose cultural knowledge was built through lived relationship with these communities, rather than training completed before deployment, are a functional requirement. The main job is building trust slowly. That cannot be contracted around.

The IHS and the VA Were Not Designed to Find Each Other

A Native American Veteran is qualified for services from both the Indian Health Service and the Veterans Health Administration, and neither system was designed to coordinate with the other. In practice, it required navigating two bureaucratic systems with separate databases, separate eligibility determinations, and separate program structures with no routine mechanism for communicating about the same patient, as documented in VA MIRECC VISN 19's 2024 analysis.

Standard Programming Assumes And Female Veterans

The VA healthcare system was built over decades during which women in the military were understood as supplementary to the primary mission. Women-specific services in rural VA facilities are frequently unavailable, and the distance to the nearest facility offering women's health programming is often greater than the distance to the nearest general clinic, which is itself already often prohibitive.

A program that mixes populations without attending to the specific dynamics that military sexual trauma creates, including the way safety, trust, and disclosure operate differently for this population, produces predictable outcomes: lower engagement, earlier dropout, and women Veterans who conclude, again, that the available help was not built for them.

There is a woman who participates in Televeda's Thursday evening virtual peer support group, Agnes Nez, from a rural county with no women's VA clinic within a hundred miles. The group draws women from Navajo County, Apache County, and homebound Veterans who would otherwise have no access to peer support of any kind. She has not been asked to disclose what happened to her. She has been asked to show up. That distinction is the reason she comes back. She did not respond to standard VA outreach and her non-response was recorded as disengagement.

IHS	≠	VA
Can reach her physically. Has no social isolation program. No referral channel to VA.		Has social isolation program. She doesn't appear in utilization data. Drive is impossible seasonally.

Many fall between two systems not because either is indifferent to their wellbeing, but because neither was designed to find them - VA MIRECC VISN 19, 2024

04

SECTION FOUR

What the Evidence Actually Shows

The field has more evidence than it uses. An honest account of what research shows: at what level of rigor, and under which conditions.

Across every format and population the literature has studied, peer connection is the active ingredient. Technology is the delivery mechanism; the peer relationship is what produces change. Any model that uses a platform without building genuine sustained community is delivering content with the appearance of community, and these do not produce the same outcomes.

✓ STRONGEST EVIDENCE BASE

In-Person Group Programming

Structured, regular group interaction reduces both subjective loneliness and objective isolation over time. Effects persist at 6- and 12-month follow-ups when programs persist. Critical limitation: evidence base is almost entirely drawn from populations who can attend in person. Rural, homebound, and tribal adults are systematically absent.

△ MODERATE, SHORT-TERM EFFECTS

One-to-One Contact

Telephone and video peer contact produces measurable short-term reductions in loneliness for homebound older adults (Masi et al., 2011). Effects are smaller than group-based interventions and do not reliably persist after the program ends. A one-to-one connection can establish a relationship but not a community and community membership is what protects health over time.

▲ GROWING EVIDENCE · MOST RELEVANT HERE

Group-Based Virtual Programming

The CONNECTED study (Eliacin et al., 2023) evaluated a peer-facilitated virtual telehealth group intervention. Phase 2 pilot: 19 Veterans, two cohorts. Showed reductions in social isolation scores, increased perceived social support, improvements in anxiety and depression. A sample of 19 cannot establish generalizability but it can establish a signal worth following, particularly because telehealth delivery removed access barriers that would have prevented participation.

⚠ EVIDENCE GAP

AI/AN-Specific Programs

The scoping review by Good et al. (2024, *Women's Health Issues*), which screened 1,508 publications on AI/AN women Veterans and retained 28, noted explicitly that most existing data came from women who already used VA services. Televeda's Navajo Nation program data represents some of the only implementation evidence currently available for this population. Program outcomes cannot be generalized beyond those actively engaged in VA services without implementation data.

Format Matters: Synchronous vs. Asynchronous

Real-time peer groups produce different outcomes than asynchronous message-based platforms. The evidence for synchronous connection is stronger, per the National Academies' 2020 consensus report. For health systems and tribal communities considering infrastructure investment, the distinction is practical: connectivity that supports synchronous video participation is the relevant standard. Connectivity that supports message delivery alone is insufficient.

> Long-term effects of virtual group programming at one, two, and five years are not well established. Most studies follow participants for three to six months. For organizations currently operating these programs, the opportunity to produce longitudinal evidence is itself a fundable outcome and belongs in the grant application. "The gap is not evidence. The evidence exists. The gap is implementation, will, and money directed at the right models."

05

SECTION FIVE

Five Years, One State, and What Implementation Evidence Looks Like

The data from Televeda's Arizona partnerships is presented as program evaluation evidence, not controlled trial findings. It is the most specific data available for these populations under these conditions.

VA MISSION DAYBREAK — 1ST PLACE

FAST COMPANY — WORLD CHANGING IDEAS 2023

AZ DEPT. HEALTH — 5-YEAR EVALUATION

In February 2023, the U.S. Department of Veterans Affairs named Televeda a first-place winner in Mission Daybreak, a $20 million grand challenge and part of the VA's ten-year strategy to end Veteran suicide, for Project Hózhó, named for the Diné concept of health and balance that anchors the program's design. Fast Company recognized it among the World Changing Ideas of 2023. Arizona's Department of Health Services, following a comprehensive five-year review, formally incorporated Televeda into its Health Improvement Plan as an effective solution for telecare access in underserved areas.

The Model Operates on Three Principles

1. **Sustained Peer Community, Not Content Delivery**
   The goal is to build the kind of ongoing peer relationships that protect health over time. Consistent groups, consistent facilitators, sufficient duration for trust to form. A group that meets for four weeks and disbands has not delivered the intervention.

2. **Cultural Adaptation as a Design Requirement, Not a Feature**
   Talking circles are built around Diné kinship practices because the design process involved Diné community leaders, healers, and narrative therapists from the start, not as consultants added after the model was built, but as co-designers of the model itself.

3. **Reach as the Foundational Design Intent**
   Through the Arizona Department of Veterans' Services' Tribal Connectivity Project, "Roger That," Televeda and ADVS are deploying Starlink internet kits across Arizona's 22 federally recognized tribes. Four tribes have already received kits: **the Gila River Indian Community, the Salt River Pima-Maricopa Indian Community, the San Carlos Apache Tribe, and the Navajo Nation.**

> *"The Arizona Department of Veterans' Services is thrilled to bring internet access to our rural and tribal communities. Partnering with community organizations to deliver this vital resource aligns perfectly with our mission."*
>
> DYLAN DALZOTTO · ASSISTANT DEPUTY DIRECTOR, PUBLIC & INTERGOVERNMENTAL AFFAIRS, ADVS

What the Program Data Shows

In 2024, Televeda launched formal Talking Circles with the Navajo Nation Veterans Administration, signing a collaborative partnership agreement in February and launching pilots in May. The six-month pilot ran from June through December 2024, with 285 Veterans engaging with the platform.

245
AI/AN Veterans directly engaged, 2023 who would not otherwise have sought mental health services

285
Veterans engaging with platform during 6-month Navajo Nation pilot (Jun–Dec 2024)

60%
Roger That participants who scheduled VA Benefits Counselor appointments by 2025

19
Veterans connected into VA system for the first time with no prior VA relationship

6-MONTH PILOT SURVEY OUTCOMES: NAVAJO NATION TALKING CIRCLES, 2024

Self-reported outcomes from pilot survey. Response rate and denominator not published in available documentation. Results reflect respondents.

"The Talking Circle provided a space for veterans to open up about personal struggles and healing. I felt the support of my fellow veterans."

NORTHERN AGENCY
VETERAN PARTICIPANT

"Veterans helping veterans has been the most effective form of support. These circles give us a space to discuss military experiences in ways family may not understand."

FORT DEFIANCE
VETERAN PARTICIPANT

"The Talking Circle helps me feel supported, like we're all helping one another climb out of the hole together."

EASTERN AGENCY
VETERAN PARTICIPANT

These are program evaluation findings and self reported participant reports. They tell us the model reaches populations that standard VA and public health programming does not, that participants engage and return, and that early outcomes are encouraging. They do not tell us how outcomes would compare against a randomized control group. Televeda's data infrastructure positions it to support independent evaluation, organizations with relevant population data should consider whether producing that evidence together serves their demographics.

06

SECTION SIX

Six Recommendations for Health System Leaders

Use the guide below to find your entry point.

Health system / managed care leader / Building the financial case → Start with Recommendations 1 and 2

Tribal health plan director / IHS administrator
Go directly to Recommendation 3

VA program coordinator
Recommendation 4 is your entry point

Federal program officer (CMS, ACL, HRSA)
Start with Recommendation 6, then return to 3 and 4

1 Screen for Social Isolation as a Standard of Care by End of 2026

The three-item UCLA Loneliness Scale and the PROMIS Social Isolation measure are both validated for clinical settings and take under five minutes to administer. Incorporate them into annual wellness visits and chronic disease management appointments. CMS Z-codes for SDOH screening now provide a billing mechanism for documenting social isolation in the
clinical encounter. Screening without a referral pathway is theater. Before any screening program launches, map available
referral options in your catchment area, including virtual programs for rural and homebound patients.
Where referral options do not exist, that gap is the planning priority.

2 Measure Outcomes That Matter, and Measure Them Consistently

Attendance figures and satisfaction scores are the easiest metrics to collect and the least useful for demonstrating health impact. Programs contracted by health systems should report changes in validated loneliness and isolation scores at baseline, three months, and twelve months minimum. Where downstream utilization data is available, track emergency department visits, readmission rates, and primary care utilization among enrolled participants. The $1,608 annual per-person cost differential documented by Flowers et al. (2017) provides a baseline for cost-benefit modeling, apply it to your own population.

3 Require Documented Cultural Adaptation Before Contracting

A social isolation program built for an urban older adult population and extended to Native American or rural Veteran communities without redesign is not serving those communities. Health systems procuring social programming for these populations should require, as a contract condition, documented evidence of community co-design: named community partners, described adaptation processes, and demonstrated facilitator cultural competency built through lived relationship rather than pre-deployment training. The absence of this documentation is itself informative.

4

Require IHS and VA Coordination for Any Program Serving Native American Veterans

A Native American Veteran is eligible for services from both systems, and neither was designed to coordinate with the other. Any program serving this population must include an explicit coordination mechanism between IHS and VA program staff as a design requirement rather than an aspiration. Tribal Veterans Representative programs, trained community liaisons with working knowledge of both VA and tribal health systems, have demonstrated in specific tribal communities that this structural gap can be bridged, per VA MIRECC VISN 19's 2024 documentation of TVR outcomes. The model is documented
and replicable. Scale is a funding and political will question. The coordination mechanism belongs in the program specification before procurement.

5

Fund the Infrastructure Before the Program

Device access and reliable internet connectivity supporting synchronous video participation are prerequisites, not amenities. 42% of Veterans residing in rural or highly rural areas enrolled in VA healthcare do not have home internet capable of supporting VA telehealth services (VA Office of Health Equity). Any initiative targeting rural, homebound, or tribally affiliated populations must budget for device lending, connectivity support, and digital navigation as line items, established before the first session is scheduled. Programs launched into populations without this infrastructure produce low enrollment and high early dropout that gets recorded as participant disengagement. That recorded outcome is program design failure. Absent these provisions, participation data cannot be interpreted as a valid measure of program effectiveness.

6

Mapping the Federal Funding Streams Before Writing the Next Grant

The following mechanisms are currently available and have been used to fund social isolation and community connection programming for older adults, Veterans, and tribal communities:

Older Americans Act (ACL)
Funds work through Area Agencies on Aging and tribal organizations

HRSA Federal Office of Rural Health Policy
Community-based health initiatives for underserved rural and tribal populations

VA Mission Daybreak
Funds suicide prevention innovations specifically targeting AI/AN Veterans

VA Community Care Program
Funds supplemental services for Veterans who cannot access VA care due to distance

ACL ARPA Social Isolation Investments
A newer and underutilized stream for organizations with an evidence base to apply. The money is not sufficient, but it is more available than most program planners realize, and identifying the gap between what exists and what you need is more productive than waiting for a funding landscape that will not change fast enough.

METHODOLOGY

This report synthesizes peer-reviewed literature, federal survey data, and government reports published between January 2020 and March 2026. Sources were identified through searches of PubMed, PsycINFO, and Google Scholar using relevant keywords, alongside document repositories from VA, IHS, ACL, HRSA, GAO, and CMS. Evidence was prioritized by methodological rigor, with emphasis on systematic reviews and meta-analyses, followed by large longitudinal studies, smaller trials, and program evaluations. Key methodological limitations are noted where relevant.

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ABOUT

TELEVEDA

Televeda is a digital health platform founded in 2018 dedicated to reducing social isolation and increasing digital inclusion for Veterans, seniors, and Native American communities.

29
Countries of operation

2018
Founded

RECOGNITION

- First-place winner, U.S. Department of Veterans Affairs Mission Daybreak Challenge $20M grand challenge for Veteran suicide prevention innovation
- Fast Company World Changing Ideas 2023
- Arizona Department of Health Services incorporated into 5-Year Health Improvement Plan as effective solution for telecare access in underserved areas

TO DISCUSS PARTNERSHIPS, PROGRAM DATA, OR IMPLEMENTATION SUPPORT:

outreach@televeda.com
televeda.com/heros-story-project